TATA 1mg | Labs®

PERSONAL HEALTH SMART REPORT

Prepared for

Mahak Jahan Aara

Basic Info

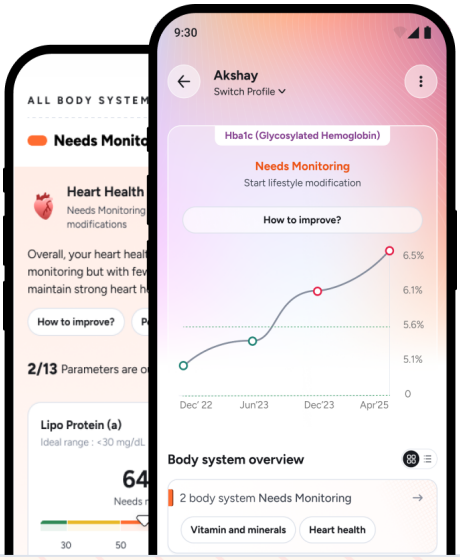
Female /20 Yrs

Sample collection date

12/03/2026

View detailed health insights, and trends on Tata 1mg app.

Click here



Doctor Summary

Urine R/M (Urine Routine & Microscopy)

Note This is an electronically generated **summary of clinically significant parameters of the attached report.** It is advised to read this summary in conjunction with the attached report and to correlate it clinically. For the trends section, the out-of-range values are highlighted with respect to the bio-reference range of respective reports.

Test Name	Result	Bio. Ref. Interval	Trends (For last three tests)		
Complete Blood Count	12 Mar 26		12 Feb 26	30 Nov 25	-
Hemoglobin	**9.2** g/dL	12.0 - 15.0	**9.2** (12.0 - 15.0)	**8.8** (12.0 - 15.0)	-
RBC	**4.95** 10^6/cu.mm	3.8 - 4.8	**4.96** (3.8 - 4.8)	4.67 (3.8 - 4.8)	-
HCT	**29.8** %	36 - 46	**29.5** (36 - 46)	**27.9** (36 - 46)	-
MCV	**60.2** fL	83 - 101	**59.6** (83 - 101)	**59.7** (83 - 101)	-
MCH	**18.6** pg	27 - 32	**18.6** (27 - 32)	**18.8** (27 - 32)	-
MCHC	**30.8** g/dL	31.5 - 34.5	**31.3** (31.5 - 34.5)	31.5 (31.5 - 34.5)	-
RDW-CV	**19.4** %	11.5 - 14	**17.2** (11.5 - 14)	**16.7** (11.5 - 14)	-
Total Leucocyte Count	9.48 10^3/ÂµL	4 - 10	8.27 (4 - 10)	**12.66** (4 - 10)	-
Neutrophils	60 %	40 - 80	65.9 (40 - 80)	72.3 (40 - 80)	-
Lymphocytes	32 %	20 - 40	26.7 (20 - 40)	20.6 (20 - 40)	-
Monocytes	06 %	2 - 10	5.4 (2 - 10)	6.2 (2 - 10)	-
Eosinophils	02 %	1 - 6	1.2 (1 - 6)	**0.9** (1 - 6)	-
Basophils	00 %	0 - 2	0.8 (0 - 2)	0 (0 - 2)	-
Absolute Neutrophil Count	5.69 10^3/ÂµL	2 - 7	5.45 (2 - 7)	**9.15** (2 - 7)	-
Absolute Lymphocyte Count	**3.03** 10^3/ÂµL	1 - 3	2.21 (1 - 3)	2.61 (1 - 3)	-
Absolute Basophil Count	**0** 10^3/ÂµL	0.02 - 0.1	0.07 (0.02 - 0.1)	**0** (0.02 - 0.1)	-

Doctor Summary

Urine R/M (Urine Routine & Microscopy)

Test Name	Result	Bio. Ref. Interval	Trends (For last three tests)		
Complete Blood Count	12 Mar 26		12 Feb 26	30 Nov 25	-
Platelet Count	**415** 10^3/ÂµL	150 - 410	389 (150 - 410)	**417** (150 - 410)	-
MPV	**12.1** fL	6.5 - 12	**12.5** (6.5 - 12)	10.6 (6.5 - 12)	-
PDW	**19.8** fL	9 - 17	**21.4** (9 - 17)	**19.8** (9 - 17)	-
Inflammatory markers	12 Mar 26		12 Feb 26	30 Nov 25	-
Erythrocyte Sedimentation Rate	**32** mm/hr	0.5 - 12	**15** (0 - 12)	**21** (0 - 12)	-
Urine Routine & Microscopy	12 Mar 26		12 Feb 26	30 Nov 25	-
Specific gravity	1.005	1.000 - 1.030	<=1.005 (1.003 - 1.035)	1.005 (1.000 - 1.030)	-
pH	6.0	5 - 8	6.0 (4.6 - 8)	7.0 (5 - 8)	-
Glucose	Negative	Negative	Negative (Negative)	Negative (Negative)	-
Protein	Negative	Negative	Negative (Negative)	Negative (Negative)	-
Ketones	Negative	Negative	Negative (Negative)	Negative (Negative)	-
Blood	Negative	Negative	Negative (Negative)	**3+** (Negative)	-
Leucocyte Esterase	Negative	Negative	Negative (Negative)	**1+** (Negative)	-
Red blood cell	0.00 /hpf	0 - 2	Nil (0 - 2)	**20.60** (0 - 2)	-

PO No :PO10001605106-230

Customer Name	: Ms.MAHAK JAHAN AARA	Collected Via	: TATA 1MG OKHLA
Age/Gender	: 20Y 2M 5D /Female	Referred By	: Dr.
Lab Visit ID	: OKH2855728	Collection Date	: 12/Mar/2026 08:51AM
Barcode ID/Order ID	: D29850210 / 16162929	Report Date	: 12/Mar/2026 01:01PM
Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Partial Report

HAEMATOLOGY

COMPLETE HAEMOGRAM

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	**9.2**	g/dL	12.0 - 15.0	Spectrophotometry (Cyanide-free)
RBC	**4.95**	10^6/cu.mm	3.8 - 4.8	Impedance
HCT	**29.8**	%	36 - 46	Calculated
MCV	**60.2**	fL	83 - 101	Calculated
MCH	**18.6**	pg	27 - 32	Calculated
MCHC	**30.8**	g/dL	31.5 - 34.5	Calculated
RDW-CV	**19.4**	%	11.5-14	Calculated
Total Leucocyte Count	9.48	10^3/µL	4 - 10	Impedance
Differential Leucocyte Count				
Neutrophils	60	%	40-80	DHSS/Microscopy
Lymphocytes	32	%	20-40	DHSS/Microscopy
Monocytes	06	%	2-10	DHSS/Microscopy
Eosinophils	02	%	1-6	DHSS/Microscopy
Basophils	00	%	0-2	Impedance/Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	5.69	10^3/µL	2 - 7	Calculated
Absolute Lymphocyte Count	**3.03**	10^3/µL	1-3	Calculated
Absolute Monocyte Count	0.57	10^3/µL	0.2 - 1	Calculated
Absolute Eosinophil Count	0.19	10^3/µL	0.02 - 0.5	Calculated
Absolute Basophil Count	**0**	10^3/µL	0.02-0.1	Calculated
Platelet Count	**415**	10^3/µL	150-410	Impedance/Microscopy
MPV	**12.1**	fL	6.5 - 12	Calculated
PDW	**19.8**	fL	9-17	Calculated

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
DHSS : Double Hydrodynamic Sequential System Flowcytometry
Calculated parameters are either derived from Impedence measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

NABL certificate and scope

MC - 5424

This test has been performed at
TATA 1MG OKHLA
Address: 2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Dr. Dhananjay Singh
MBBS, MD(Pathology)
Consultant Pathologist
Reg No: 63325

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PO No :PO10001605106-230

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HAEMATOLOGY

COMPLETE HAEMOGRAM

Test Name	Result	Unit	Bio. Ref. Interval	Method
Erythrocyte Sedimentation Rate				
Erythrocyte Sedimentation Rate	**32**	mm/hr	0-12	Capillary Photometry

Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosis; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

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CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Urine Routine & Microscopy				
Urine Routine	Automated			
Physical & Chemical Examination				
Colour	Pale yellow		Pale Yellow	Optical array
Appearance	Clear		Clear	Optical array
Specific gravity	1.005		1.000–1.030	Refractometry
pH	6.0		5.0 - 8.0	Double Indicator
Glucose	Negative		Negative	Glucose-oxidase
Protein	Negative		Negative	Tetra-bromphenol blue
Ketones	Negative		Negative	Sodium Nitroprusside
Blood	Negative		Negative	Tetramethyl benzidine
Bilirubin	Negative		Negative	Diazonium salt
Urobilinogen	Normal		Normal	Diazonium salt
Leucocyte Esterase	Negative		Negative	Carboxylic acid ester
Nitrite	Negative		Negative	Griess reaction
Ascorbic acid	Negative		Negative	Indophenol
Microscopic Examination				
Pus cells	0.00	/hpf	0.0-5.0	
Red Blood Cells	0.00	/hpf	0.0-2.0	
Epithelial cells	0.50	/hpf	0.0-5.0	
Yeast	0.00	/hpf	0.0-1.0	
Bacteria	11.20	/hpf	0.0-80.0	
Hyaline casts	0.00	/hpf	0.0-5.0	
RBC casts	0.00	/hpf	0.0-0.99	
WBC casts	0.00	/hpf	0.0-0.99	
Hyaline-granular casts	0.00	/hpf	0.0-0.99	
Fatty casts	0.00	/hpf	0.0-0.99	
Granular casts	0.00	/hpf	0.0-0.99	
Waxy casts	0.00	/hpf	0.0-0.99	
Microorganism casts	0.00	/hpf	0.0-0.99	
Calcium oxalate monohydrate crystals	0.00	/hpf	0.0-0.99	

NABL certificate and scope

This test has been performed at
TATA 1MG OKHLA
Address: 2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Kundan

Dr. Kundan Kumar
MBBS, MD (Lab Medicine)
Consultant Laboratory Medicine
Reg No- 96030

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Test Name	Result	Unit	Bio. Ref. Interval	Method
Calcium oxalate dihydrate crystals	0.00	/hpf	0.0-0.99	
Triple phosphate crystals	0.00	/hpf	0.0-0.99	
Uric acid crystals	0.00	/hpf	0.0-0.99	
Calcium phosphate	0.00	/hpf	0.0-0.99	
Cystine crystals	0.00	/hpf	0.0-0.99	
Leucine crystals	0.00	/hpf	0.0-0.99	
Tyrosine crystals	0.00	/hpf	0.0-0.99	
Amorphous Material	0.00	/hpf	0.0-0.99	

Comment:

- Urine microscopy is done in centrifuged urine specimens.
- Results should be interpreted in conjunction with clinical findings and other laboratory investigations.
- For optimum results, the sample should be a first-void, mid-stream urine collected in a clean, dry, and sterile container. Contamination from vagina, urethra, perineum may affect results.
- Presence of red blood cells may be influenced by menstruation.
- Certain medications, dehydration, or supplements may impact results.
- Microscopy findings are semi-quantitative and do not confirm infection; urine culture may be required.
- Trace proteinuria may be observed in various physiological conditions such as prolonged recumbency, strenuous exercise, or a high-protein diet.
- Interference: False-positive results for bile pigments, protein, glucose, and nitrites may occur due to disinfectants, peroxide-based cleaning agents, therapeutic dyes, vitamin C, or certain medications.
- Ascorbic acid (Vitamin C): May interfere with glucose, nitrite, bilirubin, and blood results; repeat testing is recommended after at least 10 hours or next day after stopping vitamin C.
- Blood: Small amounts of blood may not change urine colour and are detectable only by tests; false positives may occur due to infection or contamination. Occasionally strip tests may show higher value than microscopy due to lysed RBC.
- Leukocyte esterase: Results may be influenced by drugs, coloured compounds, high glucose or protein, and urine concentration; false positives can occur due to vaginal contamination or preservatives. Occasionally strip values may exceed microscopy due to lysed white blood cells or pus cells.

***** End Of Report *****

Pending Test-Protein - Random (Spot) Urine

Disclaimer:
1. The reported results based on laboratory investigation, are only for the purposes of diagnosis and should be clinically correlated and interpreted by the referring physician/ medical practitioner. For any queries relating to the reported results, you may write to our customer support team on care@1mg.com
2. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of

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CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method

particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen.
3. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
4. The patient's details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies.
5. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
6. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only.
7. Pregnant women should seek guidance from a qualified obstetrician as test parameters may vary during pregnancy

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